As a pathologist, I studied disease through a microscope.

As an epidemiologist, I studied patterns across communities.

Then it became personal.

After I lost my job, my family lost our health insurance. Just one month later, my husband was bitten by a tick and became ill with Lyme disease.

I was trained to understand disease. But as a wife, I faced another question: How do we find our way through this?

That is why I built PathwayAI.

I'm Yuwei Zhang. Let me show you.

PathwayAI follows the whole journey: preparing for time outdoors, knowing what to do after a tick bite, and finding a way into care and support.

Start with the question my family faced: What if I don't have insurance?

Here, you can find a community health center serving uninsured patients, ask about income-based fees, and get questions to ask about coverage or hospital financial assistance. Care is not automatically free, but you have a place to start—and you do not have to share your story to use it.

Now imagine someone feeling unwell, missing work, and struggling to reach an appointment. This example is fictional.

They describe their experience in their own words. PathwayAI helps organize it into a record they can check, correct, and download.

The first benefit goes to the person sharing their story.

The testing guide helps them prepare questions for a clinician. Caregivers can describe daily difficulties, with their observations clearly identified.

Here, the person paid two hundred twenty-five dollars for care and travel. They also missed three workdays. A caregiver contributed eight hours.

Money paid. Time lost. Help needed.

We keep those burdens separate. Unknown answers stay unknown.

Someone who needs transport can prepare a request about eligibility, cost, booking, and the return trip. They choose what to share.

Now we step back—from one household to the community.

Here, Dutchess and Orange counties provide a starting point. Public data show reported Lyme disease, population context, and healthcare resources.

Where might support be needed? What resources exist? What do we still need to ask patients?

Published cost research helps illustrate the possible scale of burden. The sources and assumptions stay visible. These are planning scenarios, not measured county spending.

The next question is practical: What can a community actually do?

If patients describe transport barriers, partners could review ride support. If families struggle to afford care, they could review health-center access, coverage assistance, and hospital financial aid.

But identifying a gap does not create money or appointments.

The planning brief helps partners discuss existing capacity, staffing, cost, and possible funding. It could support a county budget discussion or a grant proposal. It does not secure funding or promise savings.

Patients and community partners help choose a feasible response. Then they ask: Did people reach care? Did assistance come through? Who still needs help?

These invented records show how comparable patient reports could reveal recurring needs. Shared collection is currently off. A real pilot needs consent, privacy protections, and accessible participation. Voluntary reports cannot represent everyone.

AI can organize information—but people decide what matters.

Data can reveal patterns—but patients help us understand them.

The patient stays at the center, from a useful next step today to a better-informed community decision tomorrow.

That is PathwayAI: Making the Hidden Patient Journey Visible.

To patients, caregivers, and experts: thank you. Your experiences and guidance helped make this possible.